M9 Diazepam

Scope of Practice	EP
Classification	• Benzodiazepine
Usual Presentation	• 10mg/2ml ampule • 5mg/2.5ml rectal tube
Mechanism of Action	• Potentiates the action of the GABA neurotransmitter producing sedation and seizure inhibition
Indications	• Seizures • Sedation of agitated or combative patients
Contraindications	• Known allergy
Precautions	• Respiratory distress • Shock and/or hypotension
Side Effects	• Reduced level of consciousness • Respiratory Depression • Hypotension
Adult Dose & Administration	**Seizures** • 5mg IV **Sedation** • 5mg IV • Doses should be reduced if agitation is secondary to traumatic brain injury. **Dilution for IV/IO Administration** • In a 10ml syringe, draw up 8ml normal saline. • Add 2ml (10mg) of Diazepam. • Solution concentration is then 1mg/1ml.
Pediatric Dose & Administration	**Seizures** • 0.1mg/kg IV/IO (max 5mg) • 0.5mg/kg PR (maximum 20mg) **Dilution for IV Administration** • In a 10ml syringe, draw up 8ml normal saline. • Add 2ml (10mg) of Diazepam. • Solution concentration is then 1mg/1ml.
Use in Pregnancy and Lactation	• Pregnancy: Fetal harm has been demonstrated however, diazepam should be administered if clinically indicated. • Lactation: Safety has not been established however, diazepam should be administered if clinically indicated.
Additional Information	• Diazepam is not recommended to be given IM due to its slow onset of action. Use IM midazolam. • Benzodiazepines may cause airway, breathing and circulation compromise. • If administered, all possible monitoring should be in place. • An IV (if not available in place) should be established as soon as possible after administration.